Credit Valley: ED performance

Latest month: July 2026 · Source: Ontario Health (NACRS, CIHI)

Location: Credit Valley | Mississauga | Queensway ...

Monthly ED visits

9,961

Time to first doctor (h)

2.7

Ontario 2.0

Admitted: time in ED (h)

26.0

Ontario 17.7

Admitted: left ED within 8 h

15.4%

Ontario 28.2%

Scorecard vs Ontario and provincial targets

Measure	Site (h)	Ontario (h)	Gap (h)	% Within Target	Status	Ontario rank	vs last year (h)
Time to first doctor assessment	2.7	2.0	0.7		No target	#130 of 161	0.3
Length of stay: low urgency, sent home	3.9	3.2	0.7	60.9%	Missed	#126 of 164	0.5
Length of stay: high urgency, sent home	5.7	4.7	1.0	80.7%	Near	#138 of 163	0.3
Length of stay: admitted patients	26.0	17.7	8.3	15.4%	Missed	#147 of 154	1.0

Key takeaways

- **Both sites miss the admitted-patient target.** Across Ontario, about 28% of admitted patients leave the ED within the 8-hour target. Credit Valley (15%) and Mississauga (27%) are both below that.
- **Credit Valley is in the bottom fifth.** It ranks there province-wide on every measure. Its patient volume and mix are almost identical to Mississauga's, so the difference more likely comes from how the site runs (bed flow, staffing).
- **Mississauga improved.** Its admitted patients spent 3.9 h less in the ED in July 2026 than in July 2025.
- **Queensway is an urgent care centre.** It has no admissions, so its shorter times are not directly comparable to a full ED.

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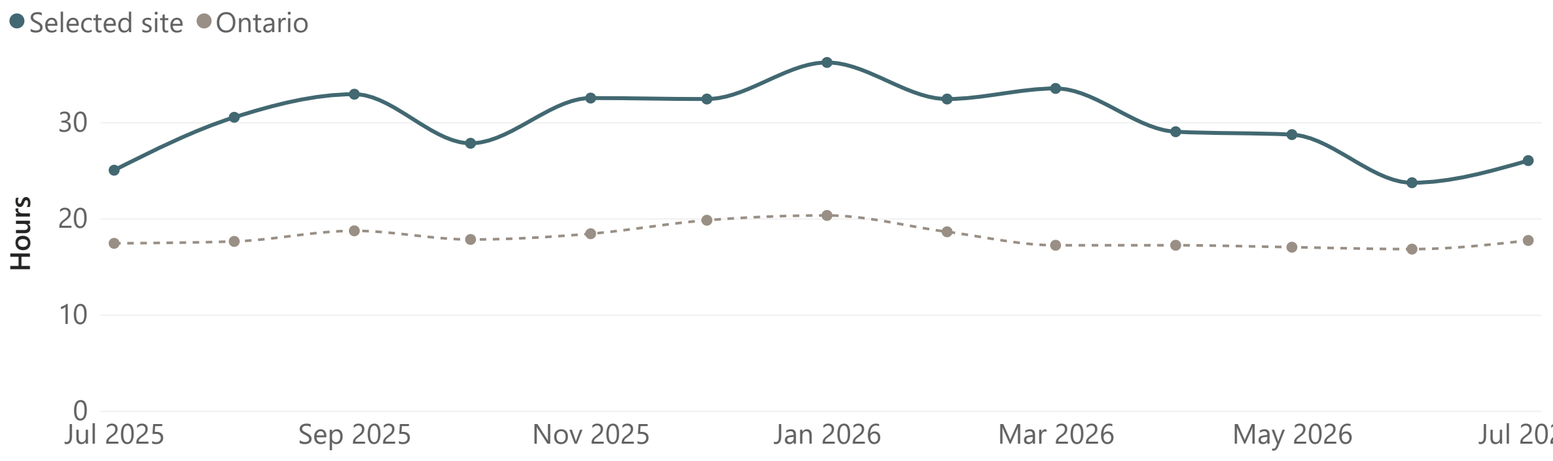
Measure

Length of stay: admitted patients

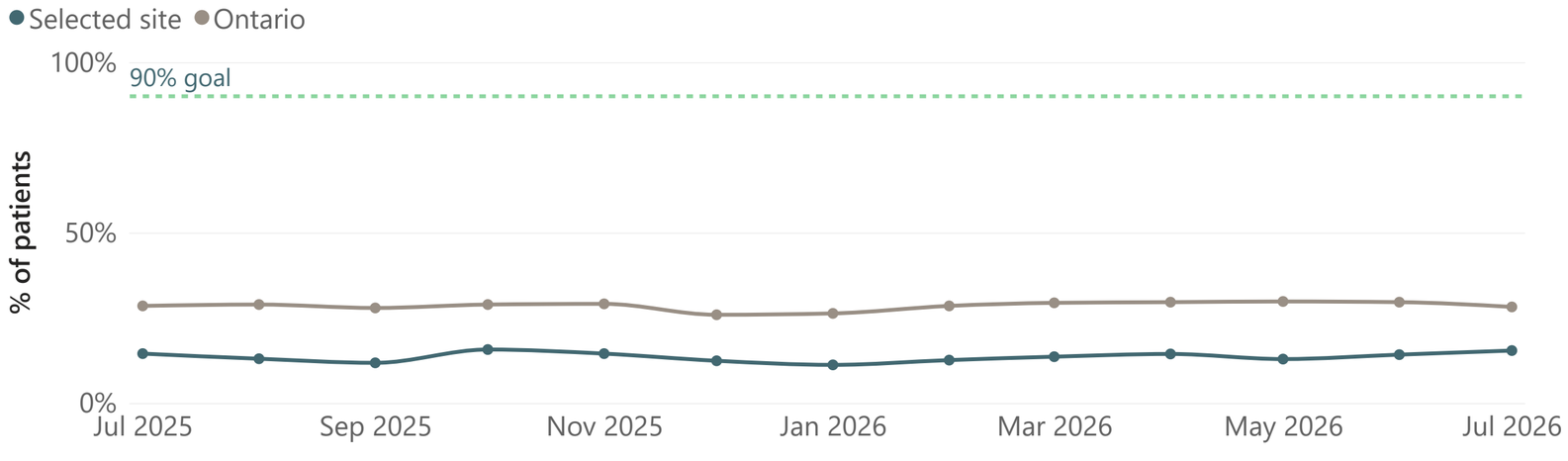
Location

Credit Valley | Mississauga | Queensway ...

Average hours: Length of stay: admitted patients



Share of patients within target: Length of stay: admitted patients



How to read this page

- **Controls:** use the dropdown to choose a measure; the site buttons choose the hospital.
- **Reference lines:** dashed grey = Ontario average. Dashed green = provincial goal (90% of patients within the target time).
- **No target for first assessment:** time to first doctor assessment has no provincial target, so the lower chart is blank for it.

What stands out

- **Mississauga improved.** It cut its admitted patients' average ED time from a peak of 32.0 h (Sep 2025) to 20.9 h (Jul 2026).
- **Credit Valley peaked in January.** Its admitted patients averaged 36.2 h in the ED in January 2026. In July 2026 it was 8.3 h above Ontario.
- **Ontario misses the goal every month.** Fewer than 1 in 3 admitted patients across Ontario left the ED within 8 hours in any month.

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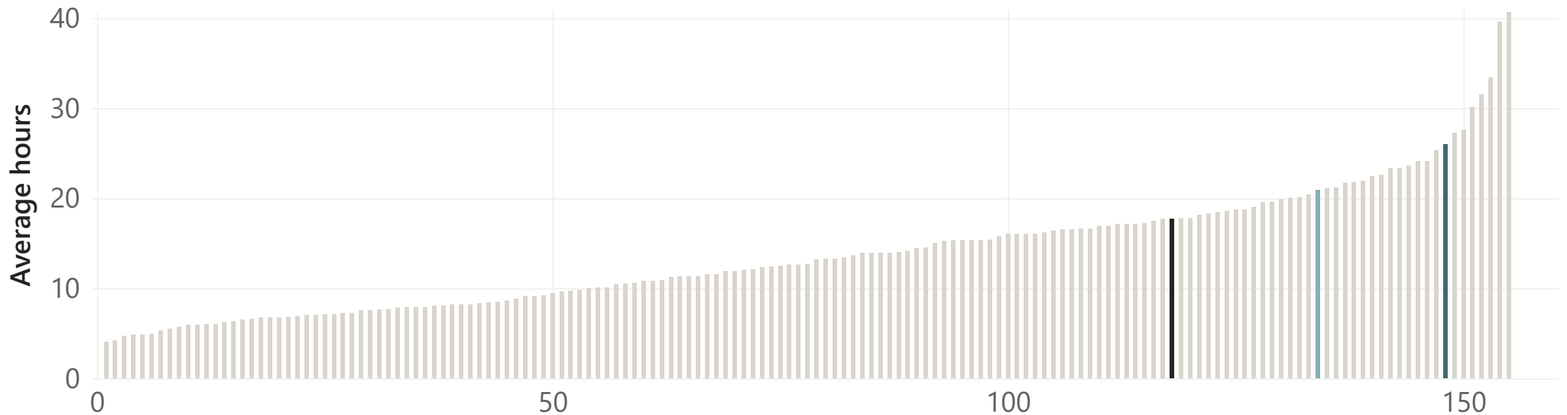
Measure

Length of stay: admitted patients

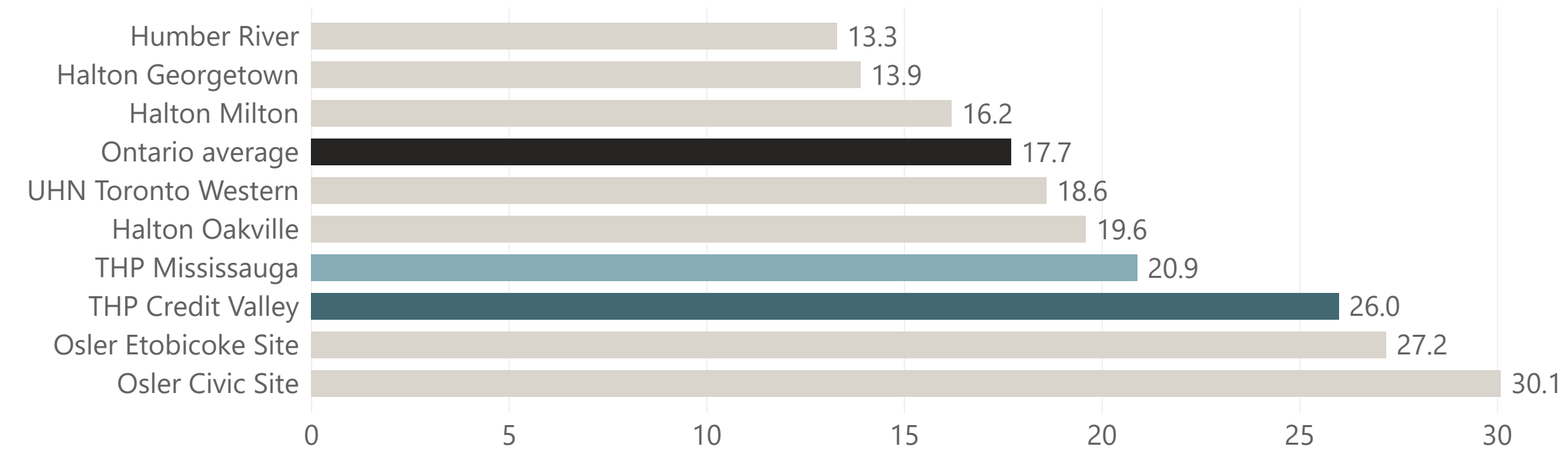
Location

Credit Valley | Mississauga | Queensway ...

Every Ontario ED, shortest to longest average wait time



Trillium vs neighbouring EDs (Peel, Halton, west Toronto)



Ontario rank (1 = shortest wait)

#147 of ...

Ontario EDs with a longer wait

5%

What stands out (admitted patients)

- **The delays are regional.** Credit Valley (26.0 h) ranks #147 of 154. The two nearby Osler sites rank lower: Etobicoke at 27.2 h (#148) and Brampton Civic at 30.1 h (#150). Long admitted-patient stays across Peel fit regional inpatient bed pressure.
- **Some neighbours beat Ontario.** Humber River (13.3 h) and Halton Milton (16.2 h) are below the Ontario average of 17.7 h.
- **Mississauga is above the median.** The median Ontario ED keeps admitted patients 12.6 h. Mississauga's 20.9 h is 8.3 h longer.
- **First-doctor ranks differ.** For time to first doctor, Osler Etobicoke (0.8 h) and Brampton Civic (1.2 h) rank in the provincial top 30; Credit Valley (2.7 h) ranks #130.

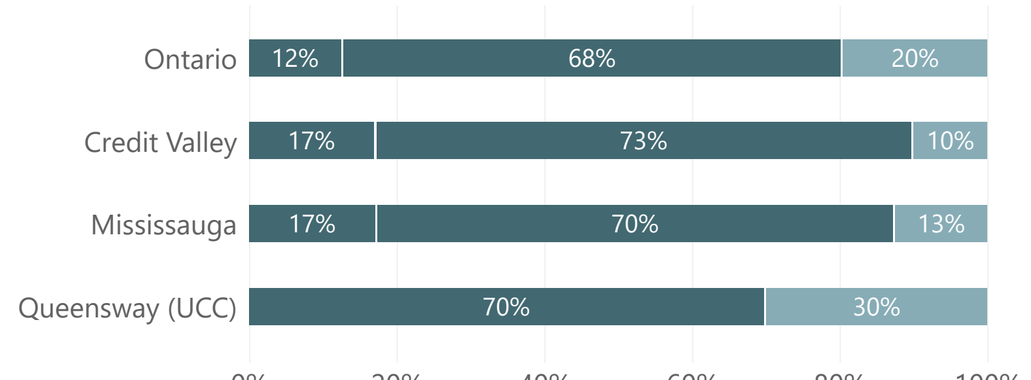
Volume & patient mix: Trillium sites vs Ontario

Latest month: July 2026 · Source: Ontario Health (NACRS, CIHI)

Measure

Time to first doctor assessment

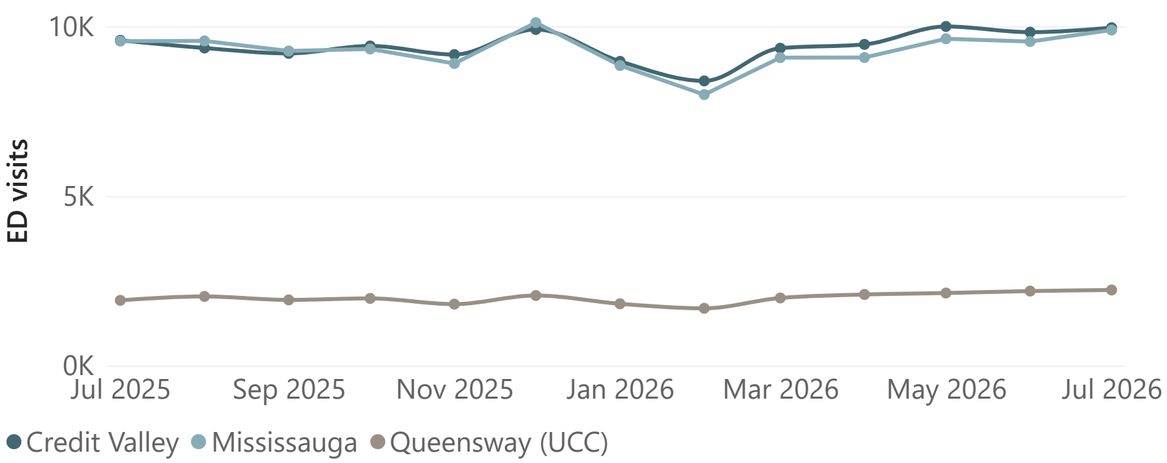
Patient mix: share of ED visits by type (13 months)



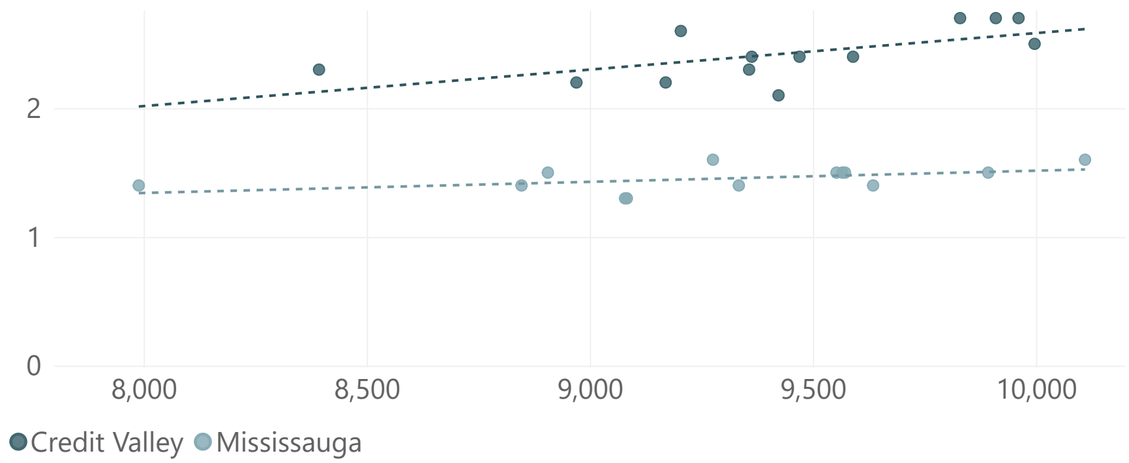
What stands out

- **Same volume and mix, longer waits.** Mississauga and Credit Valley each see about 9,300 to 9,400 ED visits a month, and about 17% of patients at each are admitted. Credit Valley waits longer on every measure, so volume and patient mix do not explain the gap. Both sites admit a larger share of patients than the Ontario average (17% against about 12%); both are large regional hospitals.
- **Busier months slow first assessment.** First-doctor waits rise in busier months (r ≈ 0.5 to 0.6). Admitted patients' time in the ED does not. That fits inpatient bed availability as the main driver of admitted-patient delays.
- **Queensway admits no one.** It sees about 2,000 visits a month, 30% of them low urgency.
- **Exploratory only.** With 13 months per site, these patterns do not prove cause.

Monthly ED visits, July 2025 to July 2026



Monthly ED visits vs average hours: Time to first doctor assessment



Methods & caveats

Data source

• **Ontario Health, "Time Spent in Emergency Departments"** public reporting, based on the National Ambulatory Care Reporting System (NACRS) from the Canadian Institute for Health Information (CIHI).
• **Trends:** monthly data from July 2025 to July 2026 (13 months) for Trillium Health Partners' three sites and the Ontario average, exported as CSV in September 2026.
• **Rankings:** the July 2026 hospital comparison table covering every Ontario ED that reports each measure (154 to 164 hospitals, depending on the measure).
• **Public data:** all data is public and aggregated. No patient-level information was used.

Measures and definitions

• **Time to first assessment:** average hours from arrival until a doctor or nurse practitioner first assesses the patient. There is no provincial target.
• **Length of stay (LOS):** average hours from arrival until the patient leaves the ED.
  – *Low urgency, sent home* (CTAS 4 to 5): target 4 h
  – *High urgency, sent home* (CTAS 1 to 3): target 8 h
  – *Admitted to hospital*: target 8 h
• **% within target:** share of patients whose stay was within the target time. The provincial goal is 90%.
• **Status:** Met = 90% or more · Near = 80 to 89.9% · Missed = under 80%. Ontario Health does not define a "Near" band; I set it.
• **Rank:** 1 = shortest average wait. Ties share a rank; Ontario isn't ranked.
• **vs last year:** July 2026 vs July 2025 (same month, to avoid seasonal effects).

How the report was built

• **Power Query:** combined 30 CSV exports into one table, split file names into site, measure and metric, converted months to dates, and reshaped the table so Ontario is its own row (533 rows).
• **Data model:** a measure lookup table (labels, targets, sort order) links the monthly trends and the provincial rankings.
• **DAX:** latest-month values, gap to Ontario, target status, provincial rank, and change from 12 months earlier.
• **Neighbouring peers:** the William Osler, Halton Healthcare, Humber River and UHN Toronto Western EDs, which serve the same Peel, Halton and west Toronto area. The choice of peers is a judgement call.
• **Validation:** key values were cross-checked against the source files and an independent Python calculation (hidden QA page). The volume and wait-time correlations were also calculated in Python.

Caveats and limitations

• **Averages hide the extremes.** A few very long stays can raise an average. Ontario Health's public data doesn't include medians or 90th percentiles for each site.
• **Queensway is an urgent care centre.** It admits no patients (reported as "NV" for admitted measures), so it is not directly comparable to a full ED.
• **Correlations are exploratory.** The volume and wait-time correlations come from 13 months per site. Bed availability fits the data, but no occupancy or staffing data was available to test it.
• **Patient mix is approximate.** The three patient groups are counted separately and add up to 100 to 102% of total visits.
• **Rankings are one month only.** They are for July 2026 and can shift from month to month.
• **Data lags about two months.** Figures are for July 2026.